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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 95 of 117



A 70-year old man is admitted with recurrent decompensated heart failure. Medical history is notable for diabetes, stage 2 chronic kidney disease, hypertension, and coronary artery disease status-post coronary artery bypass grafting 6 years ago. The patient now has ischemic cardiomyopathy with left ventricular ejection fraction of 20%. He is a current, every-day smoker. During admission, he has progressive pulmonary edema and cardiogenic shock necessitating inotropic support. His course is complicated by a stroke resulting in dysarthria with intact fine and gross motor function. During his four-week long hospitalization, he is diuresed and stabilized on milrinone at 0.375 mcg/kg/min, furosemide 80 mg oral twice a day and spironolactone 25 mg oral daily. A right heart catheterization is repeated on current milrinone infusion, revealing blood pressure 94/60 mmHg, right atrial pressure of 8 mm Hg, pulmonary artery pressure of 49/19 mm Hg with pulmonary capillary wedge pressure of 19 mm Hg. Cardiac index is 2.1 L/min/m².

At this time, he demonstrates New York Heart Association class III symptoms, is able to walk without significant dyspnea, but reports fatigue requiring a rest after ambulating 50 feet. Your team discusses next best management steps with the patient and his family. This includes a discussion of risks, benefits and candidacy for durable ventricular assist device and heart transplantation. You express that he has favorable hemodynamics for a durable left ventricular assist device however has contraindications to heart transplantation given ongoing tobacco use. The patient is able to demonstrate understanding of the recommendation for durable left ventricular assist device but is adamant about discharge home today.

Which of the following is the best next step to manage this patient

A. Discuss continued hospitalization for implantation of durable left ventricular assist device with the patient's healthcare power of attorney



B. Offer later admission for additional evaluation of advanced heart failure therapies



C. Schedule a dedicated goals-of-care discussion with the patient

D. Refer for neurocognitive evaluation

Testing Point: The Board-certified AHFTC specialist should recognize the role of shared decision making in patients with end-stage heart failure.

Correct Answer: C. Schedule a dedicated goals-of-care discussion with the patient

Rationale: This patient has end-stage heart failure secondary to ischemic cardiomyopathy. He is demonstrating inotropic dependence despite treatment for decompensated heart failure. Additionally, he has CKD and recent stroke. Goals of care should be discussed with end-stage heart failure patients, including in the evaluation for advanced therapy candidacy. The patient's desire to be discharged home after this prolonged complex hospitalization should not be used in isolation to make inferences about the patient's short- and long-term goals of care. The role of chronic home inotropes should be discussed, including a bridge to LVAD versus palliative strategy and prognosis.

The patient has capacity to make independent medical decisions and therefore answer A (contact the patient's healthcare power of attorney) should only be done with the patient's agreement and is unnecessary unless the patient wishes to activate his POA. Prior to outpatient scheduling of admission for LVAD implantation (answer B), the patient's goals of care and hesitancy to proceed with LVAD at this time should be further explored. His history of stroke and renal dysfunction should be presented as risks for post LVAD complications such as recurrent stroke or need for dialysis, to allow shared decision making with patient.

Answer D, neurocognitive testing, may be employed during advanced heart failure therapy evaluation in select patients to assist with decision making and care planning. However, a patient's desire to be discharged home should not be misinterpreted as need for neurocognitive evaluation, and should instead prompt better understanding of patient's understanding of his disease and discussion of goals of care.

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